Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0004	**Age / Sex**	68 / F
Setting	Emergency Dept / Stroke unit	**Primary dx**	Left MCA ischemic stroke

Chief complaint

Sudden right-sided weakness and speech disturbance.

History of present illness

Husband reports she was well at 07:30, then dropped her cup and could not speak clearly. EMS called at 07:45, arrival 08:25. Symptom onset (last known well) 07:30. No head trauma, no headache.

Past history / medications

Hypertension, dyslipidemia, paroxysmal AF not on anticoagulation (declined previously). Amlodipine, atorvastatin.

Examination

BP 182/101, HR 96 irregularly irregular. Right facial droop, right arm 1/5 and leg 2/5, left gaze preference, global aphasia with some comprehension deficit. NIHSS 19.

Glasgow Coma Scale

Eyes open to voice. Verbal response confused with word-finding errors and fluent but incoherent speech. Motor: localizes with the left arm, minimal movement on the right.

GCS on arrival	Score	Response
Eye opening (E)	3	To voice
Verbal (V)	4	4 - Confused
Motor (M)	5	Localizes pain
TOTAL GCS		**12**

Investigations

Non-contrast CT: dense left MCA sign, no hemorrhage, early loss of insular ribbon. CT angiography: proximal left M1 occlusion. Glucose 7.2 mmol/L, INR 1.0, platelets normal.

Assessment

Acute ischemic stroke, left MCA (M1) occlusion, likely cardioembolic. GCS 12 and NIHSS 19.

Plan / disposition

Within window at 1 h 10 min from onset: IV alteplase started, then transfer for mechanical thrombectomy. Post-procedure stroke unit, anticoagulation planned after follow-up imaging.